DRUGS · BLEEDING AND HAEMOSTASIS

Anticoagulants and antiplatelets around ENT surgery

An increasing number of ENT patients take an anticoagulant (warfarin or a direct oral anticoagulant) or an antiplatelet drug. Around surgery, the question is always the balance between bleeding and thrombosis: how high is the bleeding risk of the operation, how high is the patient's thrombotic risk, and is there a way to reduce one without raising the other. In epistaxis, stopping these drugs is usually not necessary and may be dangerous.

1 CHAPTER ONE At a glance

The drugs on one page.

1.1 Summary

ANTICOAGULANTS AND ANTIPLATELETS AROUND ENT SURGERY

	SUMMARY
Warfarin	Vitamin K antagonist; stop 5 days before surgery with an INR check (aim below 1.5, or as the surgeon requires) the day before; bridge with low molecular weight heparin only in high thrombotic risk (mechanical mitral valve, older valves, recent venous thromboembolism within 3 months, recent stroke)
DOACs	Apixaban, rivaroxaban, edoxaban (factor Xa inhibitors) and dabigatran (thrombin inhibitor); stop about 24 hours before low bleeding-risk and 48 hours before high bleeding-risk surgery with normal renal function; dabigatran needs longer (up to 4 days) as renal function falls; no bridging is needed
Heparins	Therapeutic low molecular weight heparin: last dose 24 hours before surgery; unfractionated heparin infusion: stop 4 to 6 hours before

	SUMMARY
Aspirin	Usually continued for most ENT surgery; consider stopping 7 days before procedures where bleeding is catastrophic or hard to control (for example some skull base, middle ear and microvascular operations) if prescribed for primary prevention
Clopidogrel, prasugrel, ticagrelor	Stop 5 to 7 days before high bleeding-risk surgery, but **not** within the first 6 to 12 months after a coronary stent without cardiology advice (stent thrombosis is often fatal)
Restarting	Usually 24 hours after low-risk and 48 to 72 hours after high-risk surgery, once haemostasis is secure; venous thromboembolism prophylaxis in the meantime

CHAPTER TWO

2 Using them in ENT

Practice.

2.1 Bleeding risk of ENT operations

Higher risk: tonsillectomy, sinus and skull base surgery, septorhinoplasty, thyroid and neck surgery, free flaps, and procedures in closed spaces (orbit, middle ear). Lower risk: endoscopy, minor skin surgery, grommets, nasal cautery, FNA (which can be done on anticoagulants). Agree each plan with the patient, the anticoagulation clinic and, for stents and valves, cardiology.

2.2 Epistaxis in the anticoagulated patient

Most epistaxis can be controlled without stopping the anticoagulant: treat locally (cautery, packing, absorbable materials), check the INR on warfarin, and consider reversal only for life-threatening bleeding or a markedly raised INR. ENT UK consensus discourages routine cessation, because the stroke risk of stopping often outweighs the benefit.

3 CHAPTER THREE Safety

What goes wrong.

3.1 Key interactions

Antibiotics (clarithromycin, metronidazole, co-trimoxazole, ciprofloxacin, fluconazole, miconazole gel) raise the INR on warfarin; enzyme inducers lower it. DOACs interact with strong CYP3A4 and P-glycoprotein inhibitors and inducers (azoles, clarithromycin, rifampicin, carbamazepine). NSAIDs add bleeding risk to all.

4 CHAPTER FOUR For the examination

What the examiner wants to hear.

4.1 Viva essentials

WHAT THE EXAMINER IS LOOKING FOR

- Stopping times for **warfarin, DOACs and clopidogrel**.
- Who needs **bridging**.
- The coronary stent warning.
- Management of epistaxis on anticoagulants.

4.2 Common pitfalls

COMMON PITFALLS

- Stopping clopidogrel in a patient with a recent stent without cardiology advice.
- Bridging patients on DOACs.
- Stopping warfarin for a minor nosebleed.

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